

Risk reduction of cognitive decline and dementia: WHO guidelines, second edition

Executive summary



World Health
Organization

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Executive summary

This is the second edition of the World Health Organization (WHO) guidelines on risk reduction of cognitive decline and dementia; the first edition was released in 2019. This updated edition reflects the latest evidence and advances in dementia risk reduction.

Background

Because there is no widely accessible disease-modifying treatment or cure for dementia, prevention across the life course remains the most effective strategy to reduce future incidence.

In 2021, an estimated 57 million people globally were living with dementia, over 60% of whom resided in low- and middle-income countries. Because there is no widely accessible disease-modifying treatment or cure for dementia, prevention across the life course remains the most effective strategy to reduce future incidence. The guidelines reflect a life-course understanding of dementia risk, recognizing that exposure to risk factors accumulates over time and that certain risk factors may have a greater impact at specific life stages. This perspective also acknowledges that some associations of risk factors with dementia late in life may reflect early disease processes rather than causal effects, highlighting the importance of timing in risk reduction efforts. Several dementia risk factors overlap with those of other noncommunicable diseases, underscoring the need for integrated public health approaches.

Since the first edition of the guidelines was published in 2019, evidence supporting individual-level, multidomain and population-level interventions has strengthened. These updated guidelines aim to identify interventions that effectively lower dementia risk; however, they do not replace clinical guidance for managing each specific risk factor.

Who are the updated guidelines for?

The guidelines apply to adults living without dementia, including adults with normal cognition, adults with mild cognitive impairment, and adults with normal cognition or mild cognitive impairment living with another health condition. They are intended primarily for health care providers working in first- and second-level facilities, including district-level outpatient and inpatient services. However, the guidelines may also be of use to policy-makers, for supporting population-level interventions and ensuring access to essential health services.

What's new?

In 2024, the WHO Secretariat conducted a scoping review to determine whether new evidence had emerged since the publication of the 2019 guidelines ([Annex 1](#)).

The guidelines reflect a life-course understanding of dementia risk, recognizing that exposure to risk factors accumulates over time and that certain risk factors may have a greater impact at specific life stages.

Based on this scoping review, the Guideline Development Group (GDG):

- validated existing recommendations for five risk factors – that is, the GDG reaffirmed the existing recommendations for interventions for which new evidence was considered unlikely to alter existing guidance; and
- requested comprehensive evidence syntheses for:
 - eight risk factors for which recommendations were made in the 2019 guidelines or for which evidence was insufficient at the time; for these risk factors, the GDG determined that the evidence warranted a full review, which might change the strength or content of the existing recommendation; and
 - eight new risk factors, not covered in the 2019 guidelines – that is, the GDG determined that expanded recognition of certain conditions (e.g. HIV, stroke and traumatic brain injury) as contributors to cognitive decline warranted a full review and the development of new recommendations.

Although population-level policies are important enablers of good brain health across the life course, high-quality evidence directly linking population-level policies to reduced incidence of dementia remains limited, with the exception of air pollution interventions.

Recommendations and existing guidance

The recommendations in the guidelines are organized into three groups:

- interventions promoting healthy behaviours and lifestyles for dementia risk reduction;
- treatments or management of health conditions or biological states that are associated with an increased risk of dementia; and
- interventions addressing environmental risk factors.

In the absence of direct data on dementia, evidence was inferred from cognitive functioning. When recommendations apply only to specific age groups or to individuals exposed to certain risk factors at a particular age, this is explicitly stated. If no age group is specified, the recommendations apply to all adults.

In addition to dementia-specific recommendations, for each risk factor, the updated guidelines include generic WHO guidance, with links to relevant WHO guidelines, technical tools and frameworks, as appropriate. Dementia-specific recommendations are issued in cases where there is evidence of benefit for reducing dementia risk or improving cognition that goes beyond the established benefits of treating individual risk factors.

Table A summarizes relevant generic WHO guidance (i.e. published WHO guidelines, technical documents and frameworks) and the dementia-specific recommendations.

Table A.
Summary of generic WHO guidance and dementia-specific recommendations

Interventions promoting healthy behaviours

Cognitive activity

Generic WHO guidance

Lifelong learning through early nurturing interactions, equitable access to quality education, and cognitively stimulating work and environments should be promoted for everyone, to optimize brain health (including cognitive health) across the life course, in line with WHO guidance.

Clinical practice:

- *Integrated care for older people: guidelines on community-level interventions to manage declines in intrinsic capacity (1)*

Service delivery and care pathways:

- *Nurturing care for early childhood development: a framework for helping children survive and thrive to transform health and human potential (2)*
- *Framework to implement a life course approach in practice (3)*
- *Integrated care for older people (ICOPE): guidance for person-centred assessment and pathways in primary care, 2nd ed (4)*

Public health and policy:

- *Optimizing brain health across the life course: WHO position paper (5)*

Recommendations

UPDATED RECOMMENDATION

To reduce the risk of cognitive decline and/or dementia specifically, cognitive training^a may be offered to older adults with normal cognition or mild cognitive impairment.

Strength of recommendation: conditional

Certainty of evidence: low

NEW RECOMMENDATION

To reduce the risk of cognitive decline and/or dementia specifically, engagement in cognitive stimulation^b may be encouraged.

Strength of recommendation: conditional

Certainty of evidence: very low

^a Cognitive training refers to a targeted and repetitive cognitive exercise or task that stimulates specific cognitive domains.

^b Cognitive stimulation refers to structured activities or programmes in a group or social setting, within a set amount of time, as well as everyday activities that enhance general cognitive function such as reading, storytelling and playing games that individuals can undertake themselves.

Social activity

Generic WHO guidance

Social participation is strongly connected to good health and well-being throughout life, and should be supported over the life course, in line with WHO guidance.

Clinical practice:

- No dedicated WHO clinical guideline

Service delivery and care pathways:

- *Integrated care for older people (ICOPE): guidance for person-centred assessment and pathways in primary care, 2nd ed (4)*

Public health and policy:

- *From loneliness to social connection: charting a path to healthier societies: report of the WHO Commission on Social Connection (6)*
- *Social isolation and loneliness among older people: advocacy brief (7)*

Recommendations

UPDATED RECOMMENDATION

To reduce the risk of cognitive decline and/or dementia specifically, engagement in social activity interventions may be recommended to adults with normal cognition or mild cognitive impairment.

Strength of recommendation: conditional

Certainty of evidence: very low



Physical activity

Generic WHO guidance

Regular physical activity across all ages and abilities – including aerobic movement, muscle strengthening activities and reduced sedentary behaviour – should be promoted for everyone, in line with WHO guidance.

Clinical practice:

- *WHO guidelines on physical activity and sedentary behaviour (8)*

Service delivery and care pathways:

- *Promoting physical activity through primary health care: a toolkit (9)*
- *Promoting physical activity for older people: a toolkit for action (10)*
- *WHO package of essential noncommunicable (PEN) disease interventions for primary health care (11)*

Public health and policy:

- *Tackling NCDs: best buys and other recommended interventions for the prevention and control of noncommunicable diseases (12)*

Recommendations



VALIDATED RECOMMENDATION

To reduce the risk of cognitive decline specifically, physical activity should be recommended to adults with normal cognition.

Strength of recommendation: strong

Certainty of evidence: moderate



VALIDATED RECOMMENDATION

To reduce the risk of cognitive decline specifically, physical activity may be recommended to adults with mild cognitive impairment.

Strength of recommendation: conditional

Certainty of evidence: low



Reduce the harmful use of alcohol

Generic WHO guidance

Alcohol use is associated with many health risks, owing to its psychoactive, toxic and dependence-producing properties. Hence, alcohol should not be recommended in any amount for health benefits, in line with WHO guidance.

Clinical practice:

- *Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders – specifically, the module on alcohol use disorders (13)*

Service delivery and care pathways:

- *WHO package of essential noncommunicable (PEN) disease interventions for primary health care (11)*

Public health and policy:

- *WHO Global strategy to reduce the harmful use of alcohol (14)*
- *Global alcohol action plan 2022–2030 (15)*
- *Global status report on alcohol and health and treatment of substance use disorders (16)*
- *The SAFER technical package: five areas of intervention at national and subnational levels (17)*
- *Tackling NCDs: best buys and other recommended interventions for the prevention and control of noncommunicable diseases (12)*
- *Alcohol fact sheet (18)*

Recommendations



VALIDATED RECOMMENDATION

To reduce the risk of cognitive decline and/or dementia specifically, interventions aimed at reducing or ceasing hazardous and harmful drinking (including providing support to people with alcohol dependence) may be offered to adults with normal cognition or mild cognitive impairment.

Strength of recommendation: conditional

Certainty of evidence: moderate

Tobacco cessation

Generic WHO guidance

Tobacco use should be avoided entirely. All people who use tobacco should be supported to quit through evidence-based behavioural and pharmacological cessation interventions, and all people should be protected from exposure to tobacco smoke, in line with WHO guidance.

Clinical practice:

- *WHO clinical treatment guideline for tobacco cessation in adults (19)*

Service delivery and care pathways:

- *Strengthening health systems for treating tobacco dependence in primary care to enhance skills and integrate evidence-based cessation practices into routine care (20)*
- *WHO package of essential noncommunicable (PEN) disease interventions for primary health care (11)*

Public health and policy:

- *Tackling NCDs: best buys and other recommended interventions for the prevention and control of noncommunicable diseases (12)*

Recommendations



VALIDATED RECOMMENDATION

To reduce the risk of cognitive decline and dementia specifically, interventions for tobacco cessation should be offered to adults who use tobacco.

Strength of recommendation: strong

Certainty of evidence: low

Healthy, balanced diet and nutrition

Generic WHO guidance

A healthy, balanced diet should be recommended to all adults, in line with WHO guidance.

Clinical practice:

- *Carbohydrate intake for adults and children: WHO guideline (21,22)*
- *Guideline: sugars intake for adults and children (23)*
- *Use of non-sugar sweeteners: WHO guideline (24)*
- *Total fat intake for the prevention of unhealthy weight gain in adults and children: WHO guideline (25)*
- *Saturated fatty acid and trans-fatty acid intake for adults and children: WHO guideline (26)*
- *Guideline: sodium intake for adults and children (27,28)*
- *Guideline: potassium intake for adults and children (28)*

Service delivery and care pathways:

- *WHO package of essential noncommunicable (PEN) disease interventions for primary health care (11)*

Public health and policy:

- *Healthy diet (12)*
- *Fiscal policies to promote healthy diets: WHO guideline (29)*
- *Tackling NCDs: best buys and other recommended interventions for the prevention and control of noncommunicable diseases (12)*

Recommendations



UPDATED RECOMMENDATION

To reduce the risk of cognitive decline and/or dementia specifically, a healthy, balanced dietary pattern^a may be recommended to adults with normal cognition or mild cognitive impairment.

Strength of recommendation: conditional

Certainty of evidence: moderate



VALIDATED RECOMMENDATION AGAINST

To reduce the risk of cognitive decline and/or dementia specifically, supplementation of vitamins B and E, omega-3 polyunsaturated fatty acids (PUFA) and multivitamins/minerals is not recommended.^b

Strength of recommendation: strong

Certainty of evidence: moderate

^a To be healthy, diets need to be (22):

- **adequate:** providing enough essential nutrients to prevent deficiencies and promote health, without excess;
- **balanced:** in energy intake and energy sources (i.e. fats, carbohydrates and proteins);
- **diverse:** including a wide variety of nutritious foods within and across food groups to favour nutrient adequacy and consumption of other health-promoting substances; and
- **moderate:** in consumption of foods, nutrients or other compounds associated with detrimental health effects.

The exact makeup of a diet will vary depending on individual characteristics, preferences and beliefs, cultural context, locally available foods and dietary customs.

^b This applies only to individuals without established vitamin deficiencies.

Management of health conditions and biological states

Management of obesity

Generic WHO guidance

Management of obesity should be offered to all adults^a living with obesity, in line with WHO guidance.

Clinical practice:

- *Guideline on the use of glucagon-like peptide-1 (GLP-1) therapies for the treatment of obesity in adults (30)*

Service delivery and care pathways:

- *Health service delivery framework for prevention and management of obesity (31)*

Public health and policy:

- *WHO acceleration plan to stop obesity: a joint WHO/UNICEF operational model for designing and implementing the response (32,33)*
- *Recommendations for the prevention and management of obesity over the life course (34)*
- *Global action plan for the prevention and control of noncommunicable diseases (35)*

^a WHO guidance applies to all non-pregnant adults with obesity.

Recommendations



UPDATED RECOMMENDATION

To reduce the risk of cognitive decline and/or dementia specifically, interventions for reducing midlife overweight or obesity may be offered.

Strength of recommendation: conditional

Certainty of evidence: low to moderate



NEW RECOMMENDATION

To reduce the risk of cognitive decline and/or dementia specifically, dietary restriction interventions may be offered to adults who are overweight or obese.

Strength of recommendation: conditional

Certainty of evidence: low to moderate

Management of diabetes

Generic WHO guidance

Management of diabetes should be offered to all adults with diabetes, in line with WHO guidance.

Clinical practice:

- *HEARTS D: diagnosis and management of type 2 diabetes (36)*

Service delivery and care pathways:

- *WHO package of essential noncommunicable (PEN) disease interventions for primary health care (11)*

Public health and policy:

- *The WHO Global Diabetes Compact (37)*
- *Global action plan for the prevention and control of noncommunicable diseases 2013–2020 (35)*
- *Tackling NCDs: best buys and other recommended interventions for the prevention and control of noncommunicable diseases (12)*

Recommendations



UPDATED RECOMMENDATION

To reduce the risk of cognitive decline and/or dementia specifically, management of diabetes may be offered to adults with diabetes.

Strength of recommendation: conditional

Certainty of evidence: very low



Management of hypertension

Generic WHO guidance

Management of hypertension should be offered to all adults with hypertension, in line with WHO guidance.

Clinical practice:

- *Guideline for the pharmacological treatment of hypertension in adults (38)*
- *HEARTS: technical package for cardiovascular disease management in primary health care (39)*

Service delivery and care pathways:

- *WHO package of essential noncommunicable (PEN) disease interventions for primary health care (11)*

Public health and policy:

- *Global action plan for the prevention and control of noncommunicable diseases 2013–2020 (35)*
- *Tackling NCDs: best buys and other recommended interventions for the prevention and control of noncommunicable diseases (12)*

Recommendations



UPDATED RECOMMENDATION

To reduce the risk of cognitive decline and/or dementia specifically, management of hypertension may be offered to adults with hypertension.

Strength of recommendation: conditional

Certainty of evidence: very low



Management of dyslipidaemia

Generic WHO guidance

Management of dyslipidaemia should be offered to all adults with dyslipidaemia, in line with WHO guidance.

Clinical practice:

- *Hearts: technical package for cardiovascular disease management in primary health care: risk-based CVD management (39)*

Service delivery and care pathways:

- *WHO package of essential noncommunicable (PEN) disease interventions for primary health care (11)*

Public health and policy:

- *Global action plan for the prevention and control of noncommunicable diseases (35)*

Recommendations



VALIDATED RECOMMENDATION

To reduce the risk of cognitive decline and/or dementia specifically, management of dyslipidaemia at midlife may be offered.

Strength of recommendation: conditional

Certainty of evidence: low



Management of hearing loss

Generic WHO guidance

Prevention, screening and management of hearing loss should be offered, in line with WHO guidance.

Clinical practice:

- *Package of interventions for rehabilitation: module 6: sensory conditions (40)*

Service delivery and care pathways:

- *Integrated care for older people (ICOPE): guidance for person-centred assessment and pathways in primary care, 2nd ed (4)*

Public health and policy:

- *Hearing screening: considerations for implementation (41)*

Recommendations



UPDATED RECOMMENDATION

To reduce the risk of cognitive decline and/or dementia specifically, hearing aids may be offered to adults with hearing loss.

Strength of recommendation: conditional

Certainty of evidence: low



Use of menopausal hormone therapy (MHT)

General clinical remark

MHT is primarily used for symptomatic relief of vasomotor symptoms and genitourinary syndrome of menopause, not for dementia prevention.

Recommendations

NEW RECOMMENDATION AGAINST

Menopausal hormone therapy is not recommended for specifically reducing the risk of cognitive decline and/or dementia in postmenopausal women aged 65 years and older.

Strength of recommendation: conditional

Certainty of evidence: very low

INSUFFICIENT EVIDENCE FOR RECOMMENDATION

For women aged below 65 years – including those experiencing premature ovarian insufficiency, early menopause or perimenopause – and for the specific purpose of reducing the risk of cognitive decline and/or dementia, there is currently insufficient evidence to advise for or against the use of menopausal hormone therapy.



Management of depression

Generic WHO guidance

Management of depression in the form of psychological interventions and/or antidepressants should be provided to all adults with depression, in line with WHO guidance.

Clinical practice:

- *Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders (13)*

Service delivery and care pathways:

- *mhGAP intervention guide – Version 2.0 (42)*
- *Depression module of the mhGAP training manuals (43)*
- *Depression module of the WHO Academy mhGAP training – mhGAP: Integrating mental health into primary care (44)*

Public health and policy:

- *Comprehensive mental health action plan 2013–2030 (45)*

Recommendations

INSUFFICIENT EVIDENCE FOR RECOMMENDATION

For the specific purpose of reducing the risk of cognitive decline and/or dementia, there is currently insufficient evidence to recommend the management of depression.

Management of stroke

Generic WHO guidance

Management of stroke should be provided to all adults, in line with WHO guidance.

Clinical practice:

- *Package of interventions for rehabilitation: module 3: neurological conditions (46)*

Service delivery and care pathways:

- *Framework for the care of acute coronary syndrome and stroke (47)*

Public health and policy:

- *Tackling NCDs: best buys and other recommended interventions for the prevention and control of noncommunicable diseases (12)*
- *Intersectoral global action plan on epilepsy and other neurological disorders 2022–2031 (48)*
- *Reducing the burden of stroke: strengthening prevention, acute care, rehabilitation and health system readiness (49)*

Recommendations

INSUFFICIENT EVIDENCE FOR RECOMMENDATION

For the specific purpose of reducing the risk of cognitive decline and/or dementia, there is currently insufficient evidence to recommend pharmacological interventions typically used to reduce the risk of recurrent stroke.

Management of traumatic brain injury (TBI)

Generic WHO guidance

Measures to prevent head injuries and traumatic brain injury should be consistently implemented and enforced across all populations. Prevention and management strategies should align with existing WHO guidance:

Clinical practice:

- *Package of interventions for rehabilitation: module 3: neurological conditions (46)*

Service delivery and care pathways:

- *Helmets: a road safety manual for decision-makers and practitioners (50)*
- The global concussion awareness campaign from WHO and the Fédération Internationale de Football Association (FIFA) (51)

Public health and policy:

- *Global plan for the decade of action for road safety 2021–2030 (51)*
- *Respect women: preventing violence against women (53)*

Recommendations

INSUFFICIENT EVIDENCE FOR RECOMMENDATION

For the specific purpose of reducing the risk of cognitive decline and/or dementia, there is currently insufficient evidence to recommend specific interventions after traumatic brain injury.

Management of vision impairment

Generic WHO guidance

Screening of suspected vision impairment should be offered to all adults, and management should be offered to those diagnosed with vision impairment, in line with WHO guidance.

Clinical practice:

- *Package of eye care interventions (54)*

Service delivery and care pathways:

- *Vision and eye screening implementation handbook (55)*
- *Integrated care for older people (ICOPE): guidance for person-centred assessment and pathways in primary care, 2nd ed (4)*

Public health and policy:

- *World report on vision (56)*
- *Eye care in health systems: guide for action (57)*

Recommendations

INSUFFICIENT EVIDENCE FOR RECOMMENDATION

For the specific purpose of reducing the risk of cognitive decline and/or dementia, there is currently insufficient evidence to recommend vision impairment interventions.



Management of sleep

General clinical remark

Sleep is important for overall health and well-being. Maintaining good sleep – including getting enough hours of sleep, enjoying uninterrupted sleep, and achieving deep, restorative sleep – is advised throughout life.

Recommendations



INSUFFICIENT EVIDENCE FOR RECOMMENDATION

For the specific purpose of reducing the risk of cognitive decline and/or dementia, there is currently insufficient evidence to recommend interventions to treat sleep–wake disorders.



INSUFFICIENT EVIDENCE FOR RECOMMENDATION

For the specific purpose of reducing the risk of cognitive decline and/or dementia, there is currently insufficient evidence to recommend interventions to improve sleep^a (quality or duration).

^a Certain aspects of sleep contribute to sleep quality; for example, adequate sleep duration to ensure alertness the following day, continuity through uninterrupted sleep and deep sleep that provides restorative benefits.



Management of HIV

Generic WHO guidance

Antiretroviral therapy should be initiated for all people living with HIV, regardless of WHO clinical stage and CD4 cell count, to achieve and maintain viral suppression; the goal is to reach undetectable levels of HIV. Clinical management of HIV should be in line with WHO guidance.

Clinical practice:

- *WHO updated recommendations on HIV clinical management: recommendations for a public health approach (58)*
- *WHO guidelines on the management of advanced HIV disease (59)*

Service delivery and care pathways:

- *Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach (60)*
- *Consolidated guidelines on differentiated HIV testing services (61)*

Public health and policy:

- *The role of HIV viral suppression in improving individual health and reducing transmission (62)*

Recommendations



INSUFFICIENT EVIDENCE FOR RECOMMENDATION

For the specific purpose of reducing the risk of cognitive decline and/or dementia, there is currently insufficient evidence to recommend one anti-retroviral therapy (ART) combination over another; therefore, ART regimens should be used according to disease control needs and patient preferences.

Interventions addressing environmental risk factors



Reduce exposure to air pollution

Generic WHO guidance

Air pollution is toxic and contributes substantially to the global burden of communicable and noncommunicable diseases, including dementia. Reducing exposure to air pollution, both indoors and outdoors, is critical to improving health and well-being for all people, across the entire life course, in line with guidance from WHO and other organizations:

Public health and policy:

- *WHO global air quality guidelines: particulate matter (PM_{2.5} and PM₁₀), ozone, nitrogen dioxide, sulfur dioxide and carbon monoxide (63)*
- *Integrated science assessment (ISA) for particulate matter (final report, Dec 2019) (64)*

Recommendations



NEW RECOMMENDATION

Reducing exposure to household air pollution may reduce the risk or incidence of cognitive decline and/or dementia.

Strength of recommendation: conditional

Certainty of evidence: very low



NEW RECOMMENDATION

Reducing exposure to ambient air pollution (especially PM_{2.5}) may reduce the risk or incidence of cognitive decline and/or dementia.

Strength of recommendation: conditional

Certainty of evidence: very low

Interventions addressing multiple risk factors



Multidomain interventions

General clinical remark

Dementia risk factors should be addressed holistically, because multiple risks often coexist and accumulate over the life course.

Recommendations



NEW RECOMMENDATION

To reduce the risk of cognitive decline and/or dementia specifically, tailored^a, multidomain^b interventions may be offered.

Strength of recommendation: conditional

Certainty of evidence: moderate to high

^a "Tailored" refers to customizing multidomain interventions to the specific needs and context of the individual or population. This includes selecting components that address relevant modifiable risk factors that are present in a person; adjusting intensity and delivery format; and considering cultural, socioeconomic and resource constraints. Tailoring ensures that interventions are adapted to optimize feasibility, adherence and effectiveness.

^b "Multidomain" refers to targeting multiple dementia risk factors at once.

Implementation of dementia risk reduction interventions is shaped by health system, structural and sociocultural factors that are beyond individual control and thus require action beyond behaviour change alone.

Implementation considerations

Implementation of dementia risk reduction interventions is shaped by health system, structural and sociocultural factors that are beyond individual control and thus require action beyond behaviour change alone. Cross-cutting considerations include addressing structural and policy-level barriers; multisectoral collaboration and integrated, multidomain approaches; and embedding interventions within existing programmes for primary care, noncommunicable diseases, ageing and community. Strategies should balance digital and in-person delivery, strengthen workforce capacity, support public awareness and stigma reduction, and include monitoring and evaluation. Equity considerations cut across all risk factors and should be central to all implementation strategies.

Limitations and gaps in evidence and research

Although the evidence base has increased since the first edition of the guidelines, important limitations remain. Across most risk factors, the evidence base is restricted to observational studies or only a few randomized controlled trials, short follow-up periods, and infrequent use of dementia or mild cognitive impairment as outcomes. Also, evidence is largely derived from high-income countries, with limited subgroup analyses and variable definitions of interventions and outcomes. Additionally, evidence on interactions between multiple risk factors remains limited, as does evidence on feasibility, scalability, cost-effectiveness and policy-level interventions. Addressing these gaps is essential to strengthen the evidence base, and to inform equitable, effective and context-appropriate dementia risk reduction strategies and future guideline updates.

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